

DESCRIPTION:**CREAM**

Colour : White
Fragrance : Natural

CONTENT:

Miconazole Nitrate 2% w/w
Hydrocortisone Acetate..... 1% w/w
Preservative: Chlorocresol 0.1% w/w

PHARMACODYNAMICS:

Miconazole is an imidazole antifungal agent and may act by interfering with the permeability of the fungal cell membrane. It has a wide antifungal spectrum and possesses some antibacterial activity.

Hydrocortisone is an adrenocorticoid which when applied externally will produce anti-inflammatory and anti-allergic actions in the skin. It diffuses across cell membranes and complexes with specific cytoplasmic receptors. These complexes then enter the cell nucleus, bind to DNA (chromatin), and stimulate transcription of messenger RNA (mRNA) and subsequent protein synthesis of various enzymes thought to be ultimately responsible for anti-inflammatory effects of topical application of adrenocorticoids.

PHARMACOKINETICS:

When applied externally, there is very little absorption of Miconazole Nitrate through skin or mucous membranes.

When applied externally, Hydrocortisone can be absorbed systemically and occlusion or use over extensive areas increases absorption of topical applied drug. Metabolism is mostly in the skin tissue.

INDICATION:

For the topical treatment of inflamed dermatoses where infection by susceptible organisms and inflammation coexist e.g. intertrigo, infected eczema, tinea pedis, tinea cruris, tinea corporis, tinea versicolor and cutaneous candidiasis.

RECOMMENDED DOSE:

Apply a thin film of the cream to the affected area twice daily until lesion has completely disappeared. Once the inflammatory symptoms have disappeared (after about 7 days), treatment can be continued where necessary with antifungal alone.

CONTRAINDICATION:

History of sensitivity reaction to Hydrocortisone or other topical corticosteroids or Miconazole or other imidazole antifungal agents. Tuberculosis of the skin, and viral diseases, e.g. herpes simplex, varicella, vaccinia, acne vulgaris.

WARNING AND PRECAUTIONS:

Treatment should be discontinued if irritation or sensitization develops with the use of the cream and appropriate therapy instituted. In the presence of an infection, the use of an appropriate antifungal or antibacterial agent should be instituted. If a favourable response does not occur promptly, the corticosteroids should be discontinued until the infection is controlled adequately. Any side effects that are reported following systematic use of corticosteroids, including adrenal suppression, may also occur with topical corticosteroids, especially in infants and children. Systematic absorption of topical corticosteroids will be increased if extensive body surface areas are treated or if the occlusive technique is used. Suitable precautions should be taken under these conditions or when long-term use is anticipated.

Do not use in children under 2 years of age except under the advice and supervision of a doctor.

If irritation occurs or if there is no improvement within 4 weeks, discontinue use and consult a doctor or pharmacist.

In patients on warfarin, caution should be exercised and the anticoagulant effect should be monitored (see DRUG INTERACTIONS).

DRUG INTERACTIONS:

Miconazole administered systemically is known to inhibit CYP2C9 enzyme system. Due to the limited systemic availability after topical application, clinically relevant interactions occur very rarely. In patients on warfarin which is subjected to metabolism by CYP2C9, caution should be exercised and the anticoagulant effect should be monitored (see WARNING AND PRECAUTIONS).

PREGNANCY AND LACTATION:

It should not be used in pregnant patients in large amounts or for prolonged period of time.

SIDE EFFECTS/ADVERSE REACTIONS:

Local adverse reactions reported with the use of topical corticosteroids, especially under occlusive dressings, include burning, itching, irritation, dryness, folliculitis, hypertrichosis, acneiform eruptions, hypopigmentation, perioral dermatitis, allergic contact dermatitis, maceration of the skin, secondary infection, skin atrophy, striae and miliaria.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Symptoms: Excessive prolonged use of topical corticosteroids can suppress pituitary-adrenal function resulting in secondary adrenal insufficiency.

Treatment: Appropriate symptomatic treatment is indicated. Acute hypercorticotoid symptoms are virtually reversible. Treat electrolyte imbalance, if necessary. In cases of chronic toxicity, slow withdrawal of steroids is advised.

PACKING/PACK SIZE(S):

Aluminium tubes of 15 g and 50x15 g.

JAUHI UBAT DARIPADA KANAK-KANAK

KEEP OUT OF REACH OF CHILDREN

FOR EXTERNAL USE ONLY

MANUFACTURER/PRODUCT REGISTRATION HOLDER:

DYNAPHARM (M) SDN. BHD. (65683-V)
2497, Mk. 1, Lorong Perusahaan Baru 5,
Kawasan Perusahaan Perai 3,
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